



February 23, 2026

Steven Posnack
Principal Deputy Assistant Secretary for Technology Policy
Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health Information Technology (ONC)
Department of Health and Human Services
Mary E. Switzer Building,
Mail Stop: 7033A
330 C Street SW
Washington, DC 20201
Atten: HHS Health Sector AI RFI

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Mr. Posnack:

On behalf of the American Academy of Orthotists and Prosthetists (The Academy), we appreciate the opportunity to respond to the Department of Health and Human Services' Request for Information regarding the adoption and use of artificial intelligence (AI) in clinical care.

Founded in 1970, The Academy represents certified and state-licensed orthotists, prosthetists, assistants, technicians, fitters, and researchers dedicated to advancing evidence-based orthotic and prosthetic (O&P) care. More than five million Americans live with limb loss or limb difference, and millions more rely on orthotic devices to walk, work, prevent falls, correct spinal deformities, or manage neuromuscular impairment. O&P care is not elective technology; it is essential rehabilitative infrastructure that supports mobility, independence, workforce participation, and long-term health across the lifespan. Federal policy decisions affecting innovation, reimbursement, and regulation in this field therefore carry direct implications for a substantial and medically complex patient population.

Artificial intelligence technologies have meaningful potential to improve patient outcomes, enhance clinical efficiency, and support data-informed device selection, fitting, and monitoring within O&P practice. At the same time, our members have identified important considerations that must be addressed to ensure responsible and evidence-based integration of AI into clinical workflows. The Academy stands ready to assist HHS in responsibly advancing the integration of AI into clinical care as it applies to O&P patient populations. In O&P practice, AI may be incorporated into electronic health record systems used for documentation and reimbursement, as well as embedded within the physical components of orthoses and prostheses provided to patients.

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Regulatory Framework and Patient Safety

AI applications in O&P care may include AI-supported pressure mapping, gait analysis, predictive risk modeling, device optimization algorithms, ambient documentation systems, and remote monitoring technologies. These tools may enhance device fit, reduce fall risk, mitigate skin breakdown, and support more precise clinical decision-making.

However, our members have expressed concern regarding HIPAA compliance, data governance, and the handling of protected health information when AI systems are used in clinical environments. Some members have suggested avoiding AI use altogether due to the uncertainty regarding privacy safeguards and model training practices.

Additionally, our members report apprehension regarding AI “hallucinations” – the risk of confident, yet inaccurate outputs influencing documentation, billing justification, or patient counseling.

To address these concerns, The Academy encourages HHS to:

- Provide clear, accessible guidance from the HHS Office for Civil Rights (OCR) regarding HIPAA compliance for AI systems used in clinical care.
- Direct the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC) to promote transparent data governance standards addressing model training, retention, and secondary use of PHI.
- Explore, in coordination with CMS, whether existing accreditation or quality frameworks can appropriately incorporate standards for the clinical use of AI tools.
- Encourage the FDA to develop performance validation, auditability, and explainability standards for AI-enabled clinical decision-support systems.

Regulatory clarity and enforceable safeguards will be essential to fostering clinician trust and responsible innovation. HHS may consider whether a voluntary certification framework or recognized standards program, including evaluation of existing supplier and quality standards, would improve trust and compliance in the clinical use of AI tools. AI tools used for documentation should be regulated differently than AI tools that directly influence device function or clinical decision-making.

Reimbursement and Payment Policy

Current Medicare DMEPOS payment structures are tied to HCPCS codes describing physical components of orthoses and prostheses. AI-enabled functionality, including embedded software, sensor-driven analytics, remote monitoring systems, and clinical decision-support tools, is not clearly recognized under the statutory framework established in SSA §1834(h).

While AI tools can improve documentation efficiency and assist in meeting payer medical policy requirements, current reimbursement models do not reward improvements in documentation quality, outcome tracking, or workflow efficiency.

This misalignment creates several challenges:

- AI tools that enhance safety or efficiency may not be economically sustainable.
- Clinicians bear financial risk when investing in AI-enabled technologies without clear payment pathways.
- Innovation may be discouraged despite potential long-term value to the Medicare program.

Where AI-enabled tools objectively improve device fit, reduce falls, mitigate skin breakdown, or prevent secondary complications, appropriate reimbursement alignment may support long-term Medicare savings and improved patient outcomes. The Academy encourages HHS to examine:

- Whether HCPCS coding pathways could recognize AI-enabled device components or digital augmentations integrated into orthotic and prosthetic technologies.
- How embedded software and sensor-based analytics may be appropriately incorporated into DMEPOS payment methodologies.
- Demonstration models through the Center for Medicare and Medicaid Innovation to evaluate cost savings and outcome improvements associated with AI-integrated O&P care.
- Whether certain AI-enabled documentation or monitoring tools could be considered under existing remote monitoring or digital health benefit categories, and, where clinically appropriate and consistent with statutory authority, clarify whether O&P practitioners may participate in such billing pathways.

Any modernization of reimbursement frameworks should remain aligned with patient outcomes, safety, and long-term value.

As identified in the RFI, there are “inherent flaws in legacy payment systems” that stifle innovation and access to new and emerging technologies. The Academy supports revisiting these legacy payment systems to ensure access to the “highest-value interventions for patients” whether those interventions incorporate AI-enabled functionality or not. Importantly, improvements in documentation efficiency resulting from AI-supported tools should not be interpreted as reductions in clinical labor intensity for purposes of applying productivity adjustments within the HCPCS fee schedule.

Use of AI in Claims Adjudication and Prior Authorization

The Academy is also concerned about the use of AI-enabled systems by payers in claims processing, utilization management, and prior authorization determinations. O&P treatment decisions are highly individualized and clinically nuanced. If AI tools are used to automate or influence denials without meaningful human review, patients may experience inappropriate delays or denials of medically necessary devices, resulting in preventable functional decline, falls, skin breakdown, or loss of independence.

AI should not function as a substitute for individualized clinical review, nor should it be used to circumvent established coverage standards. Accordingly, HHS should ensure that payer use of AI in claims adjudication and prior authorization is subject to robust transparency and meaningful human oversight, including:

- Requiring review of denials by a qualified clinician with relevant expertise.

- Ensuring that meaningful peer-to-peer review processes are preserved, requiring that treating clinicians have timely access to discussion with payer clinicians possessing relevant expertise before final denial of claims or prior authorization requests.
- Requiring disclosure to patients and practitioners when AI tools are used in coverage or payment determinations.
- Requiring payers to remain fully accountable for decisions made with AI-enabled tools and to provide individualized explanations.
- Monitoring for discriminatory or biased outcomes, particularly among individuals with disabilities and medically complex conditions.

Research and Evidence Development

There is a significant lack of research specific to orthotic and prosthetic populations regarding AI use in clinical practice. While AI tools are evolving rapidly, policies governing their integration should be evidence-based and population-specific.

We have identified the following needs:

- Research assessing patient and clinician perspectives on AI in O&P care.
- Evaluation of efficacy in workflow integration.
- Outcome studies examining safety, functional improvements, and cost impact.
- Federal grant opportunities to support this work.

The Academy urges HHS to support targeted research initiatives, including partnerships with patient advocacy organizations and academic centers, to ensure that AI-related policy decisions are informed by high-quality data relevant to the O&P population. The Academy encourages coordination with NIH, AHRQ, and PCORI to support implementation science and patient-reported outcomes research specific to O&P populations.

Workforce and Clinical Burden

Members report that AI tools have met or exceeded expectations as they relate to clinical documentation support, including ambient listening systems, summarization of payer policies, and drafting of encounter notes.

Orthotists and prosthetists report that documentation burden reduces time available for direct patient interaction. AI-supported documentation, when appropriately validated and compliant with privacy standards, may reduce administrative workload and allow providers to devote more time to patient care.

HHS policy should recognize both the potential benefits and the need for guardrails in this domain.

Conclusion

Artificial intelligence, if thoughtfully implemented, may also improve equitable access to orthotic and prosthetic care by supporting standardized clinical assessment, expanding remote monitoring capabilities,

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and enhancing decision-making in underserved or rural communities. HHS should ensure that AI policy frameworks promote equitable deployment so that advances in technology reduce, rather than exacerbate, disparities in access and outcomes for vulnerable populations. Realizing these benefits within the O&P profession will require:

- Clear regulatory guidance.
- Robust privacy protections.
- Transparent validation standards.
- Reimbursement frameworks aligned with value.
- Targeted research specific to the O&P population.

The Academy stands ready to collaborate with HHS to ensure that AI integration in clinical care advances patient safety, supports clinician judgment, and improves functional outcomes for individuals who rely on orthotic and prosthetic technologies.

Respectfully submitted,

A handwritten signature in black ink, appearing to read "Gerald Stark". The signature is fluid and cursive, with the first name "Gerald" written in a larger, more prominent script than the last name "Stark".

Dr. Gerald Stark, Ph.D, CPO/L, FAAOP (D)

President

American Academy of Orthotists and
Prosthetists

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